NCCP 00562 · clinician draft

Brigatinib Therapy

> **Brigatinib monotherapy · ALK-positive advanced NSCLC** Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.
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> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Brigatinib alone · lead-in and continuing tablets

Active medicines: brigatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Minimal-to-low oral emetogenic risk: antiemetic treatment as required only, with an individual prescribed choice/dose/duration and interaction review.
- No routine premedication is required by NCCP. NCIS V8 parenteral schedules should not be copied as routine daily prophylaxis for this oral medicine.
- Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.
- Do not restart or increase brigatinib yourself. A break of 14 days or more for a reason other than a side effect normally needs a new seven-day lead-in before the previously tolerated dose. A break because of a side effect follows a different recovery plan. Early lung inflammation can prevent the usual increase to 180 mg.
- Discuss fertility before treatment. Effective non-hormonal contraception is advised for women during treatment and for at least four months after the last dose; men with partners who could become pregnant need effective contraception during treatment and for at least three months afterwards. Tell the team promptly about possible pregnancy. Breastfeeding must stop during treatment.
- Avoid prolonged sun exposure during treatment and for at least five days afterwards. Use protective clothing, a hat, broad-spectrum SPF 30 or higher sunscreen and lip balm. Report severe rash or sun reaction.
- Review CPK/muscle, pancreatic, glucose, blood pressure/heart rate and liver results; separate medicine-specific management from prophylaxis.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00562 v4 · p1–7; oral guidance Table 5 page 8 and Table 8 page 13 · NCCP oral SACT antiemetic guidance V6, minimal-to-low risk (as required); NCIS V8 reviewed for scope

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required